

Nausea and Vomiting

History

- Age of patient – elderly patients usually more concerning
- When did vomiting start? How much? How often?
- Associated symptoms – abdominal pain, diarrhea, fever, chest discomfort, dizziness, syncope
- Blood in emesis (hematemesis)? If so, streaks, clots, or “coffee ground emesis?”
- Last bowel movement – When?
- Past history – ANY history of abdominal surgery? (bowel obstruction can occur years after surgery) Cardiac history? Crohn’s disease? Colitis?
- Medications – any new medications?

Physical

- Vital signs – Tachycardia? Hypotension? Hypertension?
- Concern for sepsis – EtCO₂ < 25? Concerning vital signs? Fever? Altered Mental Status?
- Breath sounds? (cardiac problems and pneumonia can cause vomiting)
- Abdominal exam – is pain elicited? Where? Is abdomen distended?
- 12 - Lead EKG – should be performed as soon as is available in any patient with nausea / vomiting who is over 40, has any cardiac risk factors, or who has any abnormal vital sign

Some serious causes of Nausea and Vomiting you must consider:

- Cardiac cause – MI, angina
- Ectopic pregnancy
- Intra-abdominal infection – appendicitis, cholecystitis (gall bladder), diverticulitis
- Mesenteric ischemia / infarct
- Pancreatitis
- Pyelonephritis (kidney infection)
- Pneumonia
- Bowel obstruction

EMT/AEMT

- Follow General Medical Care Guideline

Active Nausea or Vomiting

- **Isopropyl alcohol** – nasal vapor inhalation for 1 minute.
3 alcohol prep pads placed under nose with patient sniffing vapors- may repeat every 5 minutes

EMT/AEMT - Expanded Scope (See below Paramedic section for indications and doses)

- **Ondansetron (Zofran)** 4mg ODT or IM every 5 min (x 2 doses) - Max 8mg
- **NS/LR** 500ml IO if unable to tolerate fluid by mouth and SBP <90 systolic

Paramedic

- 12- Lead as indicated
- **NS/LR** 500 – 1000 mL IV/IO to maintain SBP > 90

Active Nausea or Vomiting

- **Ondansetron** 4mg slow ODT/IM/IV every 5 min x 2 doses (max 8mg) OR
 - **Droperidol** 0.625mg IV OR 2.5mg IM; may repeat once in 30 mins after confirming QTc <450ms on EKG (if available)
 - May not return to the fireline as Droperidol is a sedative
 - Consider **Diphenhydramine** (Benadryl) 25-50 mg PO/IM/IV for cramps or nausea associated with motion sickness (may not return to the fireline due to side effect of drowsiness)
- All Antiemetics listed can cause QTc Prolongation - EKG will be an helpful tool in assessing this interval
- Administer **Magnesium sulfate 2g IV** for :
 - QTc >450ms if administering further medication
 - OR
 - QTc >500 even if not administering further QTc prolonging medications

